

Registration / Demographics

Name	Leonard Patricks
Date of Birth	November 12, 1972
Age	50
Sex	Male
Address	62 Birch Lane, Greenshire, GH 43012
Phone	(555) 678-2345
MRN	51201473
Account Number	45870361
Insurance	HealthPlus Gold
Emergency Contact	Clara Patricks - (555) 768-0912
Encounter Date	June 28, 2023
Check-In Time	08:45
Location	Gastroenterology Associates
Attending Gastroenterologist	Dr. Sarah Edmunds, MD
Referring Provider	Dr. James Noel, MD
Chief Complaint / Indication for EGD	Persistent dyspepsia and GERD symptoms.
Past Medical History	Hypertension, Hyperlipidemia
Past Surgical History	Appendectomy in 1993
Family History	Father with colorectal cancer
Social History	Former smoker, ceased in 2010. Rare alcohol use.
Allergies	Penicillin (rash)
Problem List	Hypertension, Dyslipidemia, GERD
Current Medications	Lisinopril 20 mg daily Atorvastatin 10 mg at night Omeprazole 20 mg AM Vitamin D3 2000 IU daily
Pre-procedure Vitals	BP: 128/82 mmHg HR: 72 bpm

	Temp: 98.6 °F SpO2: 98% RA ASA Classification: II
Informed Consent	The patient has been informed of the risks, benefits, and alternatives to the procedure. All questions were answered, and verbal as well as written consent was obtained.

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Pre-Procedure Assessment & Anesthesia Record

Pre-Anesthesia Evaluation	Airway: Mallampati Class II, Cardiovascular: S1 S2 regular. Pulmonary: CTAB. NPO since 2200 hours.
Last Oral Intake	Water at 20:00 previous night.
Planned Sedation	MAC with propofol 100 mg, midazolam 2 mg bolus as needed
Anesthesia Team	Dr. Alan Priestly, MD, Anesthesiologist Mary Casher, CRNA
Anesthesia - Procedure Times	Anesthesia Start: 09:10 Scope In: 09:20 Scope Out: 09:42 Anesthesia End: 09:50 Recovery Started: 09:55
Monitoring Trends	09:20 — BP: 125/78 mmHg, HR: 70 bpm, SpO2: 97% 09:30 — BP: 122/76 mmHg, HR: 68 bpm, SpO2: 98% 09:40 — BP: 120/74 mmHg, HR: 66 bpm, SpO2: 99%
Complications	None encountered.
Post-Anesthesia Evaluation	Aldrete Score: 10/10 Disposition: Released home with his wife as escort.

Procedure Note

Esophagogastroduodenoscopy (EGD)

The procedure commenced with the patient in the left lateral decubitus position. A flexible gastroscope (model FG-223) was utilized with a bite block placed to protect the instrument. Following satisfactory sedation, the endoscope was advanced through the oropharynx with minimal resistance. The upper esophageal sphincter was traversed without difficulty.

Within the esophagus, typical mucosal anatomical landmarks were observed. At 40 cm from the incisors, we noted the Z-line, and the gastroesophageal junction (GEJ) was demarcated without stigmata of reflux or varices. Only mild linear friable erythema was indicated.

Inspection of the stomach, entries were made to the fundus, body, and antrum. Moderate insufflation highlighted well-defined rugal folds without significant hiatus. There were scattered areas of patchy erythema in the antrum, compatible with mild gastritis. The pylorus was intubated successfully.

Observing the duodenum, circumferentially measured bulb walls appeared thickened and congested with minimal inflammatory changes noted in the second part.

A maneuver into retroflexion evaluated the gastric cardia and fundus, which were found to be unremarkable apart from inesquite hyperemic areas of the cardia region.

Findings:

- Esophagus: Z-line at 40 cm without rings, mild erosive signs.
- Stomach: Antral patchy erythema suggesting mild gastritis.
- Duodenum: Mild duodenitis visualized with no ulceration.

Impression:

Findings consistent with mild erosive esophagitis and gastritis.

Diagnosis list:

- Mild Reys Erosive Esophagitis
- Slight Antral Gastritis

Interventions:

Eight cold biopsies obtained from the lower esophagus and antrum.

Procedure tolerated well.

Endoscopy team present: Dr. Sarah Edmunds (Endoscopist), Judy Redmond (RN, circulator), Mike Wilkens (Endoscopy Tech).

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Specimens and Pathology

Specimen Containers	Site	Number of Fragments	Method	Fixative	Time Sent to Lab
Container A	Distal esophagus	4	Cold forceps	Formalin	09:50
Container B	Antrum	4	Cold forceps	Formalin	09:50

Lab Report

Lab Name	Accession Number
Better Diagnostics Group	LAB-TST-22340987
Pathology Report	
Gross Description: Multiple bioptic fragments. Microscopic Description: Esophagus shows squamous mucosa with focal basal hyperplasia. Gastric mucosa from the antrum exhibits chronic inflammation. Negative for intestinal metaplasia. Final Diagnosis: Chronic mild esophagitis and gastritis; H. pylori negative.	
Pathologist Signature	Report Date/Time
Dr. Evan Littome	July 2, 2023, 15:22

After Visit Summary / Discharge Instructions

The patient exhibited no overt distress in the recovery phase, and no incidents of nausea were reported. Vitals remained stable prior to discharge with ease of side flushing tolerated for discomfort diminution.

Instructions:

- **Diet/Activity:** Resume regular meals progressively. Light activity today, avoid heavy lifting.
- **Medications:** Start omeprazole 20 mg daily, morning prior to meals for the next 4 weeks. Resume all medications post-return as critical on-plan.
- **Warning Signs:** Address GI bleeding signs promptly with medical assistance as unexpected emesis or melena enables counteractive intervention needed.
- **Follow-Up:** Outpatient follow-up in 4 weeks; confirm via phone.
- **Result Notification:** Path reports reach results this week—be on-call for follow-up discussion update.

The patient confirmed understanding of instructions and was released to escort, Clara.

Discharge RN: Linda Vauks

Note: Instruction approved at: June 28, 2023, 10:05

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Electronically signed by: Dr. Sarah Edmunds, MD • June 28, 2023, 09:55